

Scalp & hair quick screen (salon chair)

Module 1 handout. Pair with Lessons 1-2.

Before you shampoo (about 10 seconds)

- ? Part in sections under good light
- ? Note: redness, flake type, oil/build-up, tenderness, broken skin
- ? Note hair picture: shed with bulbs, breakage without bulbs, pattern thinning, patchy loss, traction bands

Common salon pictures (observe, do not diagnose)

- ? Sensitive / inflamed: tight, hot, itchy, red; soften service
- ? Flake: dry powdery vs oily stuck scale; describe what you see
- ? Build-up / oiliness: coated roots, flat styles; clarify gently when appropriate
- ? Traction clues: edge thinning, short broken hairs in pull zones, tenderness
- ? Increased shed (root): bulbs on cape; ask about recent triggers and timing (illness, stress, medication change, postpartum lag weeks to months later). TE is a training term; avoid firm chair diagnosis
- ? Breakage: snapped lengths, rough cuticle, scalp density may still look full
- ? Pattern thinning: widening parting, finer crown/temples (AGA is clinical language; use observation words in salon)
- ? Patchy loss: sudden bare areas, smooth or irregular patches = red flag

Soften the service when

- ? Scalp looks red, sore, or easily irritated
- ? Client reports burn or sting with products
- ? Traction zones are tender

Stop and refer when

- ? Patchy bald areas
- ? Sudden severe shed
- ? Weeping, broken, pustular, or rapidly spreading scalp
- ? Pain out of proportion
- ? You are unsure

Chair line (safe)

"I'm noticing [plain observation]. We'll keep today gentle. If this stays heavy, patchy, or painful, a trichologist or GP should check it."

Pathway (light touch)

Education: trichologyacademy.co.uk

Clinic care: Loz sees clients in Knutsford via the Academy loop (Academy = education; clinic = client assessment)